
Intelligence Report

A Study to Investigate Multiple Sclerosis
Relapse Prevention With mRNA-1195
Compared With Placebo in Participants Aged
18 to ≤ 55 Years

LEAD SPONSOR**ModernaTX, Inc.****NCT ID****NCT06735248****DATE GENERATED****2026-03-10**

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Summary

Generated: 2026-03-10

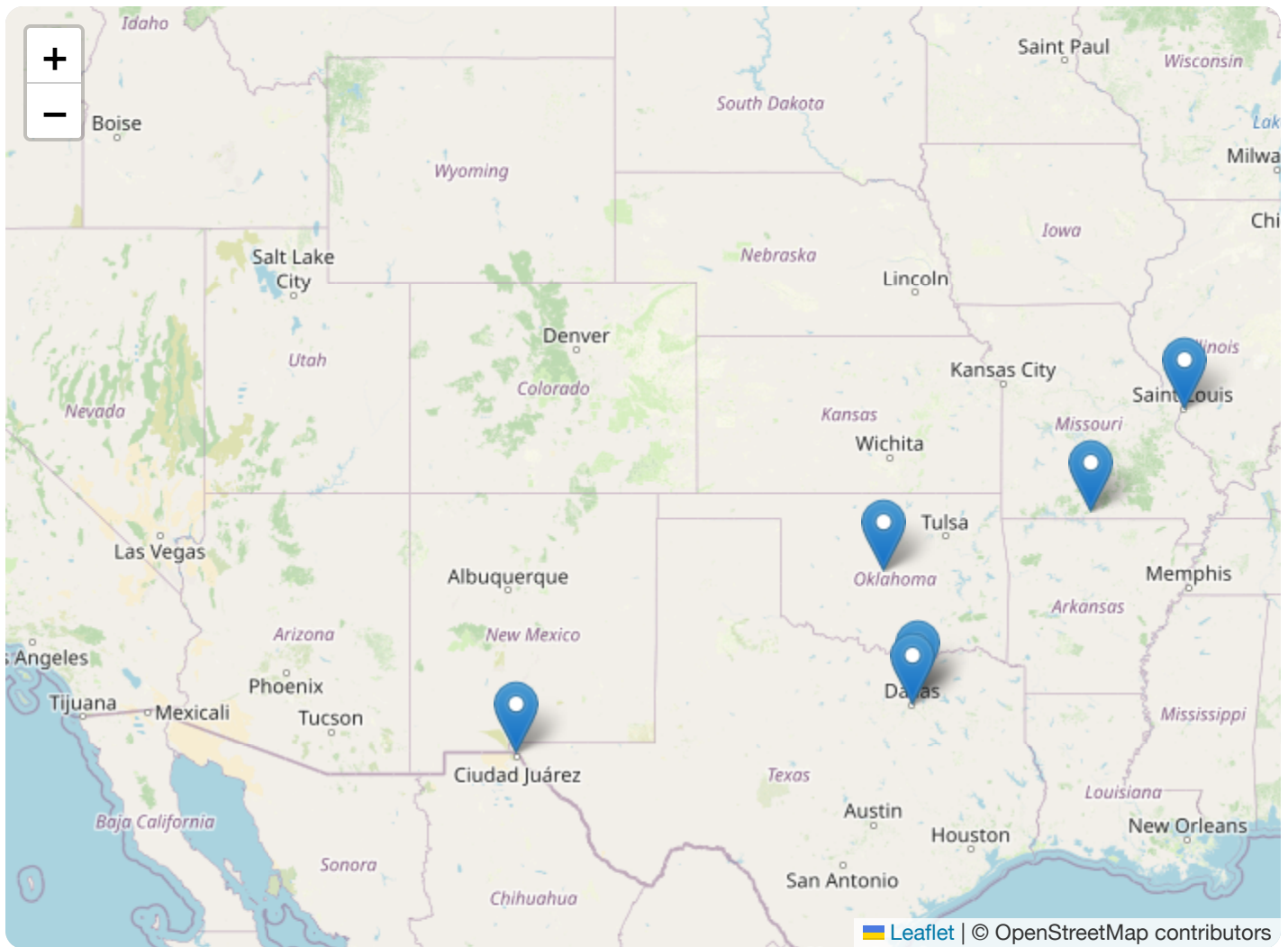
Summary	Details
Lead sponsor	ModernaTX, Inc.
Condition	Multiple Sclerosis
Study type	INTERVENTIONAL
Phase	PHASE2
Enrollment	180.0
Inclusion points	4.0
Exclusion points	7.0
Locations	17.0
Patients per site per month	0.2

- With 17 locations and an enrollment of 180 patients, your trial will maintain a recruitment rate of approximately 0.24 patients per site per month, which is higher than the 0.07-0.08 rate observed in large-scale multi-site trials like NCT07074886.
- Your trial’s 44.8-month duration is significantly more efficient than several related studies, such as NCT03109288 (136.8 months) and NCT03846219 (130.2 months), which target similar patient populations.
- By utilizing 17 sites for 180 patients, your trial will achieve a site density of 10.59 patients per site, positioning it between the high-density single-site models (e.g., NCT05961644 with 188 patients/site) and the low-density multi-site models (e.g., NCT06564311 with 1.98 patients/site).
- Your trial’s inclusion/exclusion criteria profile (4 inclusion and 7 exclusion points) is more streamlined than the average related trial, potentially facilitating faster patient screening compared to studies like NCT05961644, which requires 10 inclusion and 25 exclusion points.

✓ ClinicalTrials.gov extracted study record.

Diagnostics: Stored report loaded.

Map



- ✓ ClinicalTrials.gov extracted site records.
- ✓ OpenStreetMap basemap.

Similar trials

☐ Include only trials with at least 1 US site

Showing Phase PHASE2 interventional trials for multiple sclerosis, studying a drug, Recruiting or Not yet recruiting.

NCT ID	Lead sponsor	Enrollment	Sites	Duration	Patients per Site per Month	Map
NCT03207464	Brigham and Women's Hospital	19.0	1.0	110.7	0.2	View
NCT07074886	Hoffmann-La Roche	182.0	41.0	59.6	< 0.1	View
NCT05961644	Institute of Psychiatry and Neurology, Warsaw	188.0	1.0	60.9	3.1	View
NCT05395624	Ashvattha Therapeutics, Inc.	65.0	3.0	38.9	0.6	View
NCT05378100	Johns Hopkins University	110.0	1.0	45.0	2.4	View
NCT05418010	Queen Mary University of London	40.0	2.0	59.0	0.3	View
NCT07222956	Moein Amin	20.0	1.0	57.0	0.4	View
NCT03362294	Mapi Pharma Ltd.	30.0	7.0	104.7	< 0.1	View
NCT06170970	Johns Hopkins University	46.0	1.0	33.0	1.4	View
NCT03109288	National Institute of Allergy and Infectious Diseases (NIAID)	250.0	1.0	136.8	1.8	View
NCT05359653	University of California, San Francisco	74.0	1.0	22.1	3.3	View
NCT06564311	Zenas BioPharma (USA), LLC	93.0	47.0	17.3	0.1	View
NCT06330077	Assistance Publique - Hôpitaux de Paris	60.0	2.0	39.0	0.8	View
NCT02676739	Sarah Morrow	180.0	3.0	102.5	0.6	View
NCT07220252	TG Therapeutics, Inc.	240.0	0.0	87.0	N/A	View
NCT06592352	Clinical Academic Center (2CA-Braga)	64.0	1.0	24.2	2.6	View
NCT05092191	Centre hospitalier de l'Université de Montréal (CHUM)	250.0	1.0	30.0	8.3	View
NCT06463743	State University of New York at Buffalo	30.0	1.0	20.3	1.5	View
NCT03846219	Immunic AG	210.0	38.0	130.2	< 0.1	View
NCT06800404	Hope Biosciences Research Foundation	10.0	1.0	20.1	0.5	View
NCT06599307	Cairo University	46.0	1.0	21.0	2.2	View
NCT04267926	VA Office of Research and Development	45.0	1.0	72.0	0.6	View
NCT04121468	The Hospital for Sick Children	20.0	1.0	91.3	0.2	View

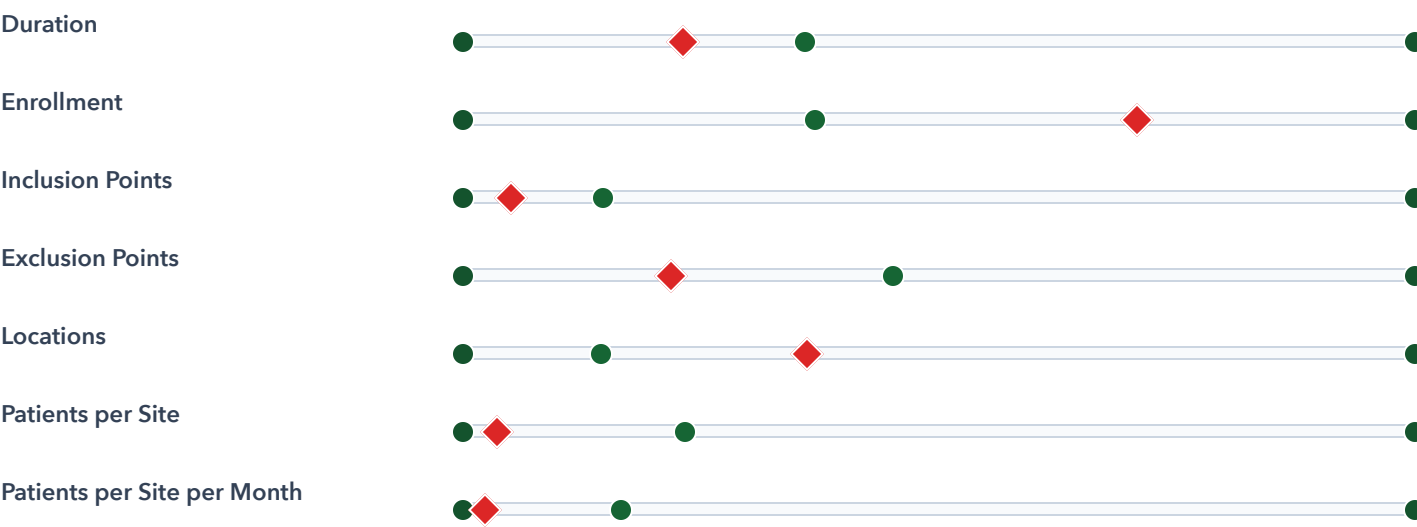
- ✓ ClinicalTrials.gov extracted study records.
- ✓ Zlick-related-trial matching and saved comparison dataset.

Key metrics comparison

☐ Include only trials with at least 1 US site

Similar trials found: 23 ?

Metric	My Trial	Min	Average	Max
Duration	44.8	17.3	60.1	136.8
Enrollment	180.0	10.0	98.8	250.0
Inclusion Points	4.0	1.0	9.8	61.0
Exclusion Points	7.0	0.0	14.5	32.0
Locations	17.0	0.0	6.8	47.0
Patients per Site	10.6	2.0	60.0	250.0
Patients per Site per Month	0.2	< 0.1	1.4	8.3



Legend: green circle = average, red diamond = My Trial, end circles = min/max.

- ✓ ClinicalTrials.gov extracted study records.
- ✓ Zlick-derived benchmark calculations from matching trials.

Closest competitors

Showing competing trials with sites in the same city than my trial.

NCT ID	Lead sponsor	Site overlap ?	Map
NCT06564311	Zenas BioPharma (USA), LLC	23.5%	Link
NCT05418010	Queen Mary University of London	11.8%	Link

US sites only

In-site competition

Showing which of my sites are also running competing trials.

No overlapping active competitors found across My Trial US sites.

- ✓ ClinicalTrials.gov extracted study records.
- ✓ Zlick-derived city-level overlap and facility matching across active related trials.

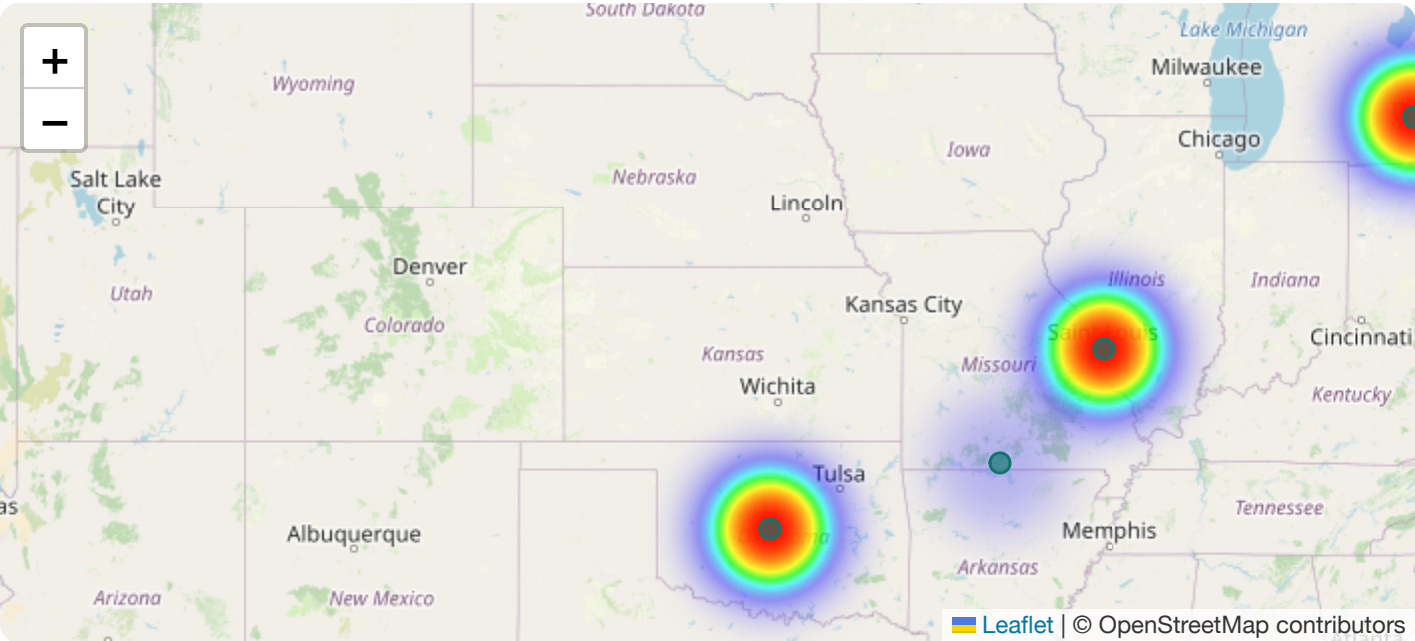
Multiple Sclerosis research nearby | Industry-sponsored

Showing PIs conducting relevant research near your sites. It also shows who they're working with and whether those sponsors have trials competing with yours.

Year: 2024 | Specialty lane: Neurology / neuro-rehab

Closest site	Provider	Specialty	Distance	Total payments	Entities	Top payer	Competitor overlap
Quest Research Institute	AARON ELLENBOGEN	Psychiatry & Neurology, Neurology	On site	\$1,773,711	40	Eli Lilly and Company	TG Therapeutics, Inc.
	MARTIN BELKIN	Psychiatry & Neurology, Neurology	On site	\$454,322	20	TG THERAPEUTICS, INC.	Hoffmann-La Roche, TG Therapeutics, Inc.
	JONATHAN FELLOWS	Psychiatry & Neurology, Neurology	On site	\$106,954	20	Biogen, Inc.	TG Therapeutics, Inc.
	JONATHAN DOTY	Psychiatry & Neurology, Neurology	12 mi	\$15,528	19	Genzyme Corporation	TG Therapeutics, Inc.
	MINA ESSAK	Psychiatry & Neurology, Neurology	On site	\$1,201	18	AbbVie Inc.	TG Therapeutics, Inc.
Oklahoma Medical Research Foundation (OMRF) MS Center of Excellence	CHELSEA BERKLEY	Psychiatry & Neurology, Neurology	On site	\$50,127	15	EMD Serono, Inc.	TG Therapeutics, Inc.
	ABEERA ALI	Psychiatry & Neurology, Neurology	On site	\$575	13	AbbVie Inc.	TG Therapeutics, Inc.
	BAHA ABU-ESHEH	Psychiatry & Neurology, Neurology	92 mi	\$13,537	18	Harmony Biosciences LLC	None
	NIDHIBEN ANADANI	Psychiatry & Neurology, Neurology	On site	\$2,032	5	Roche Products Limited	None
	JAMES BATTISTE	Psychiatry & Neurology, Neurology	On site	\$471	4	Novocure Inc.	None
Boston Clinical Trials Inc - Internal Medicine	MYRIAM ABDENNADHER	Psychiatry & Neurology, Neurology	On site	\$7,805	5	Philips North America LLC	None
	ROBERT AIKEN	Psychiatry & Neurology, Neurology	205 mi	\$5,492	3	Karyopharm Therapeutics Inc.	None

Closest site	Provider	Specialty	Distance	Total payments	Entities	Top payer	Competitor overlap
Washington University School of Medicine Neurology Clinical Unit	MADHUR BATRA	Physical Medicine & Rehabilitation, Pain Medicine	On site	\$514	3	AbbVie Inc.	None
	RINA BLOCH	Physical Medicine & Rehabilitation	On site	\$117	1	Piramal Critical Care	None
	EDMOND BENEDETTI	Physical Medicine & Rehabilitation	On site	\$40	2	Ipsen Biopharmaceuticals, Inc	None
	BARBARA GREEN	Psychiatry & Neurology, Neurology	On site	\$77,893	17	Celgene Corporation	TG Therapeutics, Inc.
	KSENIJA KOS	Psychiatry & Neurology, Neurology	On site	\$3,004	31	AbbVie Inc.	TG Therapeutics, Inc.
	WILLIAM LOGAN	Psychiatry & Neurology, Neurology	On site	\$316,604	21	NOVARTIS PHARMACEUTICALS CORPORATION	None
	GHAZALA HAYAT	Psychiatry & Neurology, Neurology	On site	\$16,143	4	Alexion Pharmaceuticals, Inc.	None
	ANTHONY MARGHERITA	Physical Medicine & Rehabilitation	On site	\$620	9	Lilly USA, LLC	None



- ✓ CMS Open Payments 2024 payments grouped by covered recipient and reporting entities
- ✓ CMS Open Payments 2024 payments grouped by covered recipient and nature of payments
- ✓ Condition-relevant provider search from CMS NPI Registry

Specialty lane: Neurology / neuro-rehab | Candidate NPIs scanned: 221

- ✓ CMS Open Payments 2024 payments grouped by covered recipient and reporting entities
- ✓ CMS Open Payments 2024 payments grouped by covered recipient and nature of payments
- ✓ Condition-relevant provider search from CMS NPI Registry

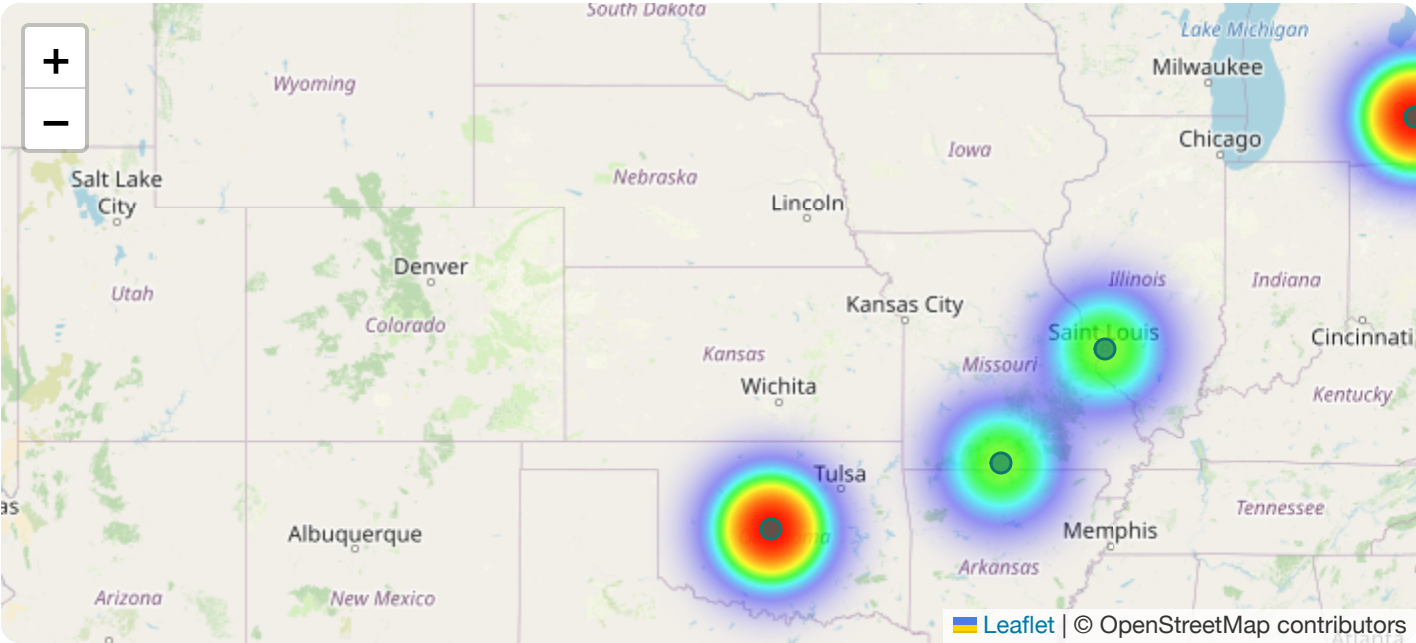
Multiple Sclerosis research nearby | NIH/public-funded

Showing research centers conducting relevant studies near your sites (funded by NIH).

Condition query: Multiple Sclerosis

Closest site	Institution	Lead PI	Recent grants	Total NIH funding	Latest FY	Example project
Boston Clinical Trials Inc - Internal Medicine	BRIGHAM AND WOMEN'S HOSPITAL	MICHAEL H. CHO	21	\$15,233,078	2026	Uncovering the genetically-driven differential susceptibility to chronic obstructive pulmonary disease and pulmonary fibrosis
	MASSACHUSETTS GENERAL HOSPITAL	Stephen N. Gomperts	10	\$8,245,105	2026	PET imaging of alpha-synuclein pathology
	BOSTON UNIVERSITY MEDICAL CAMPUS	Benjamin L Wolozin	4	\$7,009,267	2026	The role of N6-methyladenosine modified RNA in Alzheimer's disease
	BOSTON CHILDREN'S HOSPITAL	Kathleen Donovan	7	\$4,411,310	2026	Neuronal Primary Cilia Dysfunction in Tuberous Sclerosis Complex
	BETH ISRAEL DEACONESS MEDICAL CENTER	TOWIA A. LIBERMANN	4	\$3,315,525	2026	AD/ADRD and biological aging proteomic signatures in the etiopathology of delirium and its associated long-term cognitive decline
Quest Research Institute	UNIVERSITY OF MICHIGAN AT ANN ARBOR	James J. Moon	40	\$17,688,404	2026	Non-invasive and Long-lived CNS Delivery of Treg-inducing Cytokine Depots
	MICHIGAN STATE UNIVERSITY	Lane A. Baker	5	\$1,925,218	2025	An Enhanced Neurovascular Model with Scanning Conductance Imaging of Cell Junctions to Determine the Role of Matrix Metalloproteases in Multiple Sclerosis
	VAN ANDEL RESEARCH INSTITUTE	Russell Graham Jones	2	\$1,126,222	2025	Metabolic targeting of CD4 T cells in CNS autoimmunity
	HENRY FORD HEALTH SYSTEM	SHAILENDRA GIRI	2	\$705,469	2023	Endogenous metabolite restricts GM-CSF signaling pathway in pathogenic macrophages to ameliorate CNS Autoimmunity
	OAKLAND UNIVERSITY	G. RASUL CHAUDHRY	3	\$450,000	2025	Efficacy of NSCs in an EAE model of MS
Oklahoma Medical Research Foundation (OMRF) MS Center of Excellence	OKLAHOMA MEDICAL RESEARCH FOUNDATION	Robert C Axtell	13	\$7,768,926	2026	Effects of tobacco smoking on prognosis and immune function in multiple sclerosis
	UNIVERSITY OF OKLAHOMA HLTH SCIENCES CTR	Hala Chaaban	10	\$2,806,031	2026	Prevention of Necrotizing Enterocolitis

Closest site	Institution	Lead PI	Recent grants	Total NIH funding	Latest FY	Example project
Sharlin Health & Neurology	TRANSTIMULATION RESEARCH, INC.	Jiande Chen	2	\$1,258,232	2024	AccelBand, a leg-worn transcutaneous neuromodulation device for treating neurogenic bowel dysfunction in individuals with spinal cord injury
	OKLAHOMA CITY VA MEDICAL CENTER	Robert Hal Scofield	2	\$0	2023	Autoimmunity in Post-Traumatic Stress Disorder
	WASHINGTON UNIVERSITY	MARCO COLONNA	58	\$58,697,585	2026	Ontogenetic niche of B cells at the CNS borders in homeostasis, aging and autoimmunity
	SAINT LOUIS UNIVERSITY	SUSAN A FARR	1	\$502,411	2025	Fingolimod and Ozanimod for the treatment and prevention of chemobrain
	ST. LOUIS VA MEDICAL CENTER	ROBERT L WHITE	1	\$0	2025	Diffusion Basis Spectrum Imaging for Measurement of Neuroinflammation in Parkinson Disease
	ST. LOUIS COLLEGE OF PHARMACY	Ream Al-Hasani	2	\$869,391	2023	Peripheral Mechanisms of Kappa Opioid Receptor-Mediated Cold Hypersensitivity
	KINIMMUNE, INC.	Marcus Laird Forrest	1	\$395,595	2022	Development of a tumor-retentive immunostimulant as adjunct therapy for solid tumor cancers
	ST. LOUIS VA MEDICAL CENTER	ROBERT L WHITE	7	\$0	2025	Diffusion Basis Spectrum Imaging for Measurement of Neuroinflammation in Parkinson Disease



- ✓ NIH RePORTER API v2 projects search
 - ✓ My Trial US site footprint from ClinicalTrials.gov extracted records
- Condition query: Multiple Sclerosis | Institutions matched: 20
- ✓ NIH RePORTER API v2 projects search



My Trial US site footprint from ClinicalTrials.gov extracted records

Competitive timeline



- ✓ ClinicalTrials.gov extracted study records.
- ✓ Zlick-derived timeline model from study start and completion dates.

Prevalence signal

County-level prevalence heatmap for Multiple Sclerosis, with optional overlay of My Trial US sites.

Direct condition-level prevalence data is not available for this trial condition. Proxy-based fallback geography was suppressed to avoid misleading hotspot output. Previous note: Direct condition-level prevalence data unavailable. Showing state-level prevalence fallback. Proxy used: Obesity prevalence (generic chronic-disease burden proxy when direct condition data is unavailable). Secondary attempts: CDC WONDER: No direct machine-query state prevalence endpoint configured for this automated workflow. | NHANES (NCHS): No state/county prevalence granularity for this automated map workflow.

Scoring prevalence signal, competitor density, referral support, CDC social vulnerability, HRSA shortage, and county-level Census access data.

Geography: state | Source: CDC PLACES (state fallback) | Year: 2023

Direct condition-level prevalence data is not available for this trial condition. Proxy-based fallback geography was suppressed to avoid misleading hotspot output. Previous note: Direct condition-level prevalence data unavailable. Showing state-level prevalence fallback. Proxy used: Obesity prevalence (generic chronic-disease burden proxy when direct condition data is unavailable). Secondary attempts: CDC WONDER: No direct machine-query state prevalence endpoint configured for this automated workflow. | NHANES (NCHS): No state/county prevalence granularity for this automated map workflow.

- ✓ CDC PLACES prevalence data.
- ✓ ClinicalTrials.gov extracted My Trial US site records.

Protocol burden

Burden score: **56.5** (Lower burden than similar trials). Score compares protocol complexity vs similar trials using duration and inclusion/exclusion criteria.

Factor	My Trial	Similar Trial Average	Difference
Study duration (months)	44.8	60.1	-25.5%
Inclusion criteria count	4.0	9.8	-59.3%
Exclusion criteria count	7.0	14.5	-51.7%

Scale: 100 = equal to similar-trial average burden; >100 = higher burden than peers; <100 = lower burden than peers.

-  ClinicalTrials.gov extracted study record.
-  Zlick-derived comparison against saved similar trials.

Predicted funnel

Eligibility-only US feasibility score (1 = very easy, 10 = very difficult).

Score: 5.9/10

Attrition funnel (US, MS-only)

Step	Assumption	Remaining pool	Remaining %
US MS population baseline	Approximate US population living with MS.	1000000	100.0%
Disease subtype filter	RRMS-focused trials use a lower eligible share than all-MS studies.	700000	70.0%
Age filter (all ages)	Age-window attrition using MS-heavy age bands (20-50 peak).	616000	61.6%
Recent diagnosis requirement (24 months)	Early-course enrollment narrows the treatable pool.	277200	27.72%
Biomarker requirement (Epstein-Barr virus (EBV))	Biomarker-positive constraints reduce eligible prevalence.	249480	24.95%
Steroid washout (30 days)	Washout windows create timing friction for enrollment.	184615	18.46%
Common-comorbidity exclusions	Excluding frequent comorbidities trims real-world eligibility.	151384	15.14%
Criteria complexity (11 points)	Long criteria sets increase screen-failure risk.	131402	13.14%
Behavioral/operational friction	Visit/procedure burden reduces willingness to enroll.	91981	9.2%
Competitive pressure adjustment	Active US MS trials recruiting or opening soon: 0.	89222	8.92%

Top factors

- Recent diagnosis requirement (24 months): reduces estimated eligible pool by 55.0% at this step.
- Disease subtype filter: reduces estimated eligible pool by 30.0% at this step.

- Behavioral/operational friction: reduces estimated eligible pool by 30.0% at this step.
- Steroid washout (30 days): reduces estimated eligible pool by 26.0% at this step.
- Common-comorbidity exclusions: reduces estimated eligible pool by 18.0% at this step.
- Criteria complexity (11 points): reduces estimated eligible pool by 13.2% at this step.
- Age filter (all ages): reduces estimated eligible pool by 12.0% at this step.
- Biomarker requirement (Epstein-Barr virus (EBV)): reduces estimated eligible pool by 10.0% at this step.
- Competitive pressure adjustment: reduces estimated eligible pool by 3.0% at this step.

Restrictiveness signals

- Relapsing subtype requirement detected.
- Recent-diagnosis window requirement detected.
- Biomarker-positive requirement detected (Epstein-Barr virus (EBV)).
- Steroid washout requirement detected.
- Common-comorbidity exclusions detected.

Warnings

- Excluded from quantification: Medically stable as determined by Investigator
- Excluded from quantification: History of a diagnosis or condition that, in the judgment of the Investigator, is clinically unstable
- Could not quantify exclusion criterion: Clinically unstable conditions

- ✓ [National MS Society - MS prevalence](#)
- ✓ [National MS Society - prevalence news/research summary](#)
- ✓ [MSAA - About MS](#)
- ✓ [ClinicalTrials.gov API v2](#)

Modeled US pool after filters: 89,222 from 1,000,000 (8.92%). Competition estimate (active/open US MS trials): 0. Confidence: Medium.

Actual enrollment speed

Calculated using patients enrolled, number of sites, and start/end dates for similar trials reported as 'completed'.

Patients per site per month	Value
My Trial	0.2
Trials included	53.0
Mean	0.9
50th percentile	0.4
75th percentile	1.2
Minimum	0.0
Maximum	5.6

AI commentary generated successfully. Excluded invalid rows: 2. Excluded pre-2010 rows: 216.

The data distribution for patients-per-site-per-month exhibits a significant right skew, as evidenced by a mean (0.8975) that is more than double the median (0.4127). This discrepancy indicates that a small subset of high-performing sites is inflating the average, while the majority of the 53 sites are operating at a much lower recruitment velocity. The presence of extreme outliers, reaching a maximum of 5.6 compared to the 75th percentile of 1.25, confirms that the mean is not a representative measure of typical site performance.

To improve future recruitment planning, stakeholders should prioritize the median (0.4127) as the baseline expectation for site productivity rather than the mean. We recommend conducting a root-cause analysis on the top-performing sites to identify replicable best practices, while simultaneously evaluating the 218 excluded sites to determine if systemic barriers—such as site selection criteria or geographic saturation—contributed to their lack of contribution.

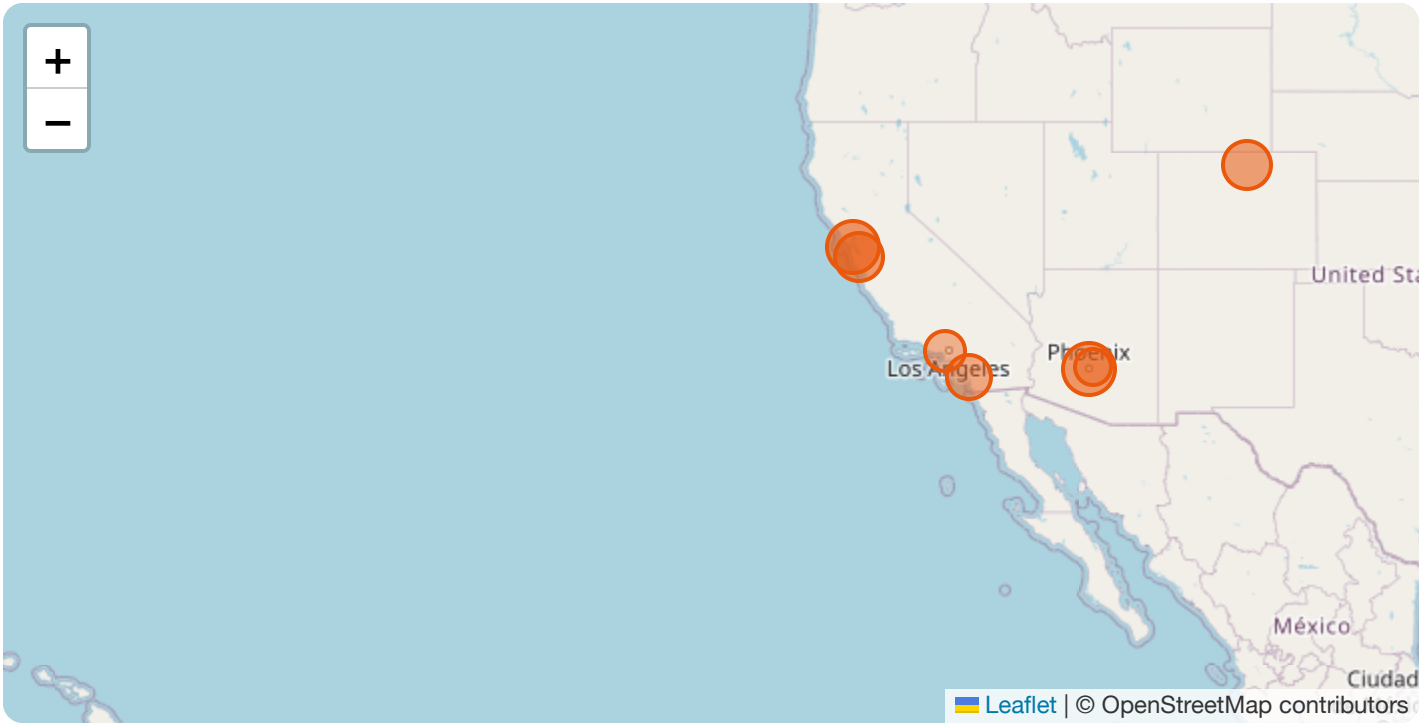
In plain English: My Trial's estimate (0.24) is below the completed-trial average (0.90), so the target appears conservative versus historical enrollment speed.

- ✓ ClinicalTrials.gov extracted study records for completed matching trials.
- ✓ Zlick-derived patients-per-site-per-month calculations.

Areas for opportunity

Beta version: Multiple Sclerosis only; US only.

i Scoring prevalence signal, competitor density, referral support, CDC data, HRSA data, and county-level Census data to identify areas where new sites are most attractive.



Scope: Multiple Sclerosis | United States | Access friction considers uninsured, no-vehicle, linguistic-isolation, CDC social vulnerability, and HRSA shortage data.

City	State	Potential referrer	Opportunity score
San Francisco	CA	AHMED ABDELHAK	High
Phoenix	AZ	DIANA ABDULRAHMAN	High
Stanford	CA	GREGORY ALBERS	High
Fort Collins	CO	TIMOTHY ALLEN	High
Carlsbad	CA	MATTHEW AUDIA	Neutral
Naples	FL	LISA AENLLE	Neutral
West Hollywood	CA	CAMERON ADAMS	Neutral
Scottsdale	AZ	AMELIA ADCOCK	Neutral
Jacksonville	FL	DAVID ABARBANEL	Neutral
Baltimore	MD	ORWA ABOUD	Neutral

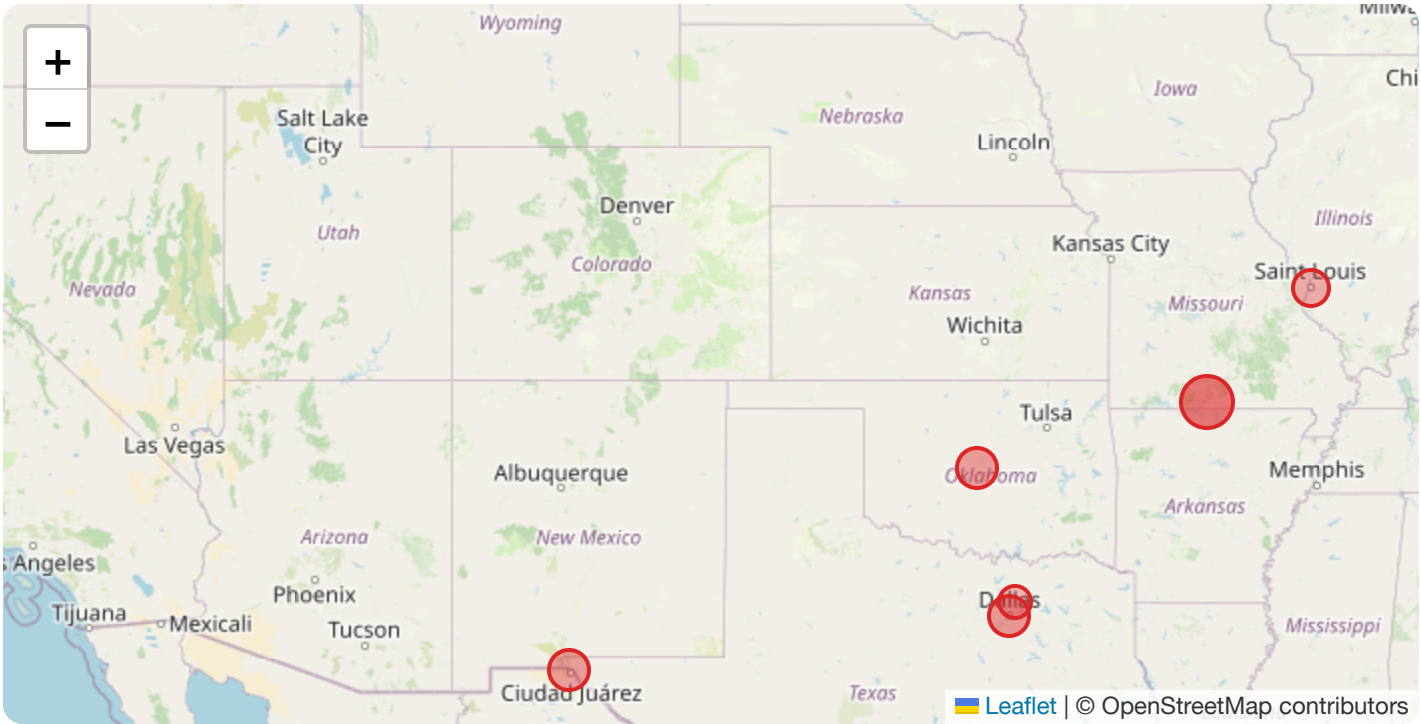
- ✓ ClinicalTrials.gov related-trial site geographies
- ✓ CDC PLACES
- ✓ CMS NPI Registry
- ✓ U.S. Census Bureau ACS 5-year profile API

- ✓ U.S. Census Geocoder
- ✓ CDC/ATSDR Social Vulnerability Index
- ✓ HRSA Health Center Program data
- ✓ HRSA HPSA data

MS-only beta. Scoring prevalence signal, competitor density, referral support, county-level ACS access data, CDC social vulnerability, and HRSA shortage and health-center signals. SVI warnings: CDC SVI county service not reachable

Areas of risk

i Scoring prevalence signal, competitor density, referral support, CDC data, HRSA data, and county-level Census data to find sites most likely to enroll slower than expected.



Scope: Multiple Sclerosis | United States | Access friction considers uninsured, no-vehicle, linguistic-isolation, CDC social vulnerability, and HRSA shortage data.

Facility	City	State	Risk score
Sharlin Health & Neurology	Ozark	MO	High
University of Texas Southwestern Medical Center	Dallas	TX	Neutral
Oklahoma Medical Research Foundation (OMRF) MS Center of Excellence	Oklahoma City	OK	Neutral
ANESC Research	El Paso	TX	Neutral
Quest Research Institute	Farmington Hills	MI	Neutral
Washington University School of Medicine Neurology Clinical Unit	St Louis	MO	Neutral
Saturn Research Solutions	Plano	TX	Neutral
Boston Clinical Trials Inc - Internal Medicine	Boston	MA	Low

- ✓ [ClinicalTrials.gov related-trial site geographies](#)
- ✓ [CDC PLACES](#)
- ✓ [CMS NPI Registry](#)
- ✓ [U.S. Census Bureau ACS 5-year profile API](#)
- ✓ [U.S. Census Geocoder](#)
- ✓ [CDC/ATSDR Social Vulnerability Index](#)
- ✓ [HRSA Health Center Program data](#)

✓ [HRSA HPSA data](#)

MS-only beta. Scoring prevalence signal, competitor density, referral support, county-level ACS access data, CDC social vulnerability, and HRSA shortage and health-center signals. SVI warnings: CDC SVI county service not reachable

Find referring centers

Support enrollment in your sites with patient referrals from nearby specialists and treatment centers.

Selected site: Boston Clinical Trials Inc - Internal Medicine - Boston, MA | ZIP: N/A

My site	Referrer	Type	Distance
Boston Clinical Trials Inc - Internal Medicine	BRAIN, SPINE AND PAIN INSTITUTE OF MASSACHUSETTS PLLC	Psychiatry & Neurology, Neurology	4.8 mi
Boston Clinical Trials Inc - Internal Medicine	ASSOCIATED PHYSICIANS OF HARVARD MEDICAL FACULTY PHYSICIANS AT BETH IS	Psychiatry & Neurology, Neurology	11 mi
Boston Clinical Trials Inc - Internal Medicine	SHABBIR ABBASI	Psychiatry & Neurology, Neurology	38 mi
Boston Clinical Trials Inc - Internal Medicine	JOSNA ADUSUMILLI	Psychiatry & Neurology, Neurology	39 mi
Boston Clinical Trials Inc - Internal Medicine	PEGAH AFRA	Psychiatry & Neurology, Neurology	39 mi
Boston Clinical Trials Inc - Internal Medicine	SEEMA ADHAMI	Psychiatry & Neurology, Neurology with Special Qualifications in Child Neurology	39 mi
Boston Clinical Trials Inc - Internal Medicine	ALBANY MEDICAL COLLEGE	Psychiatry & Neurology, Neurology	136 mi
Boston Clinical Trials Inc - Internal Medicine	ALBANY MEDICAL COLLEGE	Psychiatry & Neurology, Neurology	139 mi
Boston Clinical Trials Inc - Internal Medicine	ALBANY MEDICAL COLLEGE	Psychiatry & Neurology, Neurology	143 mi
Boston Clinical Trials Inc - Internal Medicine	ROBERT AIKEN	Psychiatry & Neurology, Neurology	205 mi

Derived ZIP: N/A | Specialty lane: Neurology / neuro-rehab | Specialty filter: Neurology, Psychiatry & Neurology, Clinic/Center, Neurology

- ✓ CMS NPI Registry.
- ✓ My Trial site location derived from ClinicalTrials.gov extracted site records.

Recommendations

[Generate new](#)

Action plan generated from the trial profile, benchmarks, competitive pressure, recruitment opportunity/risk, eligibility burden, and nearby research/referral evidence.

1. Add a site in the San Francisco Bay Area, leveraging the identified neurology provider network (e.g., Ahmed Abdelhak, Kwame Adjepong) and nearby academic institutions.

High

San Francisco ranks as the top recruitment-add opportunity (ADD score 62.0, catchment score 79.5) with 12 identified neurology providers and 2 validation signals, yet the nearest current trial site is 993 miles away. The Boston area's NIH research network (14 institutions, 60 projects near Boston Clinical Trials) demonstrates that academic-dense markets drive enrollment; San Francisco offers a comparable research ecosystem. Adding this site would also help close the severe patients-per-site-per-month gap (0.24 vs peer 1.4) by tapping an underserved geography.

2. Implement targeted referral activation at Sharlin Health & Neurology (Ozark, MO) to mitigate its high recruitment risk, or prepare a contingency replacement site.

High

Sharlin Health & Neurology carries the highest risk score (74.8) of all current sites, with only 1 identified referring provider (Kenneth Sharlin), zero validation count, zero competition count, and a catchment score of just 26.9. The trial's patients-per-site-per-month rate is already 83% below peer average (0.24 vs 1.4), meaning underperformance at even one site materially threatens enrollment timelines. The sponsor should either rapidly build a local referral network or identify a backup site in a stronger catchment area.

3. Launch a digital pre-screening campaign targeting recently diagnosed MS patients (within 24 months) who are EBV-seropositive, using MS patient registries and neurology practice EHR alerts.

High

The eligibility funnel shows the 24-month recent-diagnosis requirement is the single largest bottleneck, eliminating an estimated 55% of otherwise eligible patients, and the trial's overall eligibility score is a restrictive 5.9 out of 10. Since ~90% of MS patients are EBV-

seropositive (per the funnel's biomarker step), pre-screening can efficiently focus on recency of diagnosis. The Horizon Trial's own patient-facing site ([[thehorizontrial.com](https://www.thehorizontrial.com/)] (<https://www.thehorizontrial.com/>)) already supports online pre-screening; amplifying this through MS community channels and neurologist EHR flags at high-provider-count markets (e.g., Dallas with 12 providers, Oklahoma City with 12 providers) would accelerate identification of the narrow eligible pool.

4. Engage the Quest Research Institute (Farmington Hills, MI) PI network—particularly Aaron Ellenbogen and Martin Belkin—to activate their extensive industry relationships for cross-referral into this trial.

Medium



Quest Research Institute has the largest Open Payments footprint among trial sites (\$2.44M across 16 providers), with Ellenbogen alone receiving \$1.77M from 40 entities and Belkin \$454K from 20 entities, indicating deep industry trial experience. Nearby University of Michigan holds 40 NIH-funded neuroscience projects (\$17.7M), including MS-specific work on CNS autoimmunity. Despite this infrastructure, Quest's risk score is 49.3 with zero validation count, suggesting these relationships have not yet been activated for this specific trial. Formal co-referral agreements with these KOLs could substantially boost the site's per-month enrollment rate.

5. Establish a site in Phoenix, AZ to capture the Southwest patient population currently >345 miles from the nearest active site.

Medium



Phoenix ranks second on the recruitment-add opportunity list (ADD score 61.6) with 12 neurology providers identified and only 1 competing MS trial in the market. The trial currently has 17 sites but a patients-per-site-per-month rate 83% below peer benchmarks, indicating that geographic coverage gaps—not site count—are the binding constraint. Phoenix's large population base in Maricopa County and moderate competitive pressure make it a high-value addition, particularly given the protocol's lower-than-peer burden (burden score 56.5), which should support faster site activation.